Clinical Study Summary Report (CSSR) Document Status: Final | **Version:** 1.0 | **Date:** 06-Apr-2026

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1. Administrative & Study Identification

Full Study Title: A Randomized, Controlled, Multicenter, Open-label Trial Comparing a Hospital Post-discharge Care Pathway Involving Aggressive LDL-C Management That Includes Inclisiran With Usual Care Versus Usual Care Alone in Patients With a Recent Acute Coronary Syndrome (VICTORION-INCEPTION) **Brief Title:** Management of LDL-cholesterol With Inclisiran + Usual Care Compared to Usual Care Alone in Participants With a Recent Acute Coronary Syndrome **Short Title/Acronym:** VICTORION-INCEPTION **Protocol Number:** V-INCEPTION **NCT Number:** NCT04873934 **Trial Status:** COMPLETED (Study has been completed) **Sponsor Name / Company:** Novartis Pharmaceuticals **Investigational Product:** Inclisiran sodium (Leqvio) 300 mg **ATC Code:** C10AX16 **Phase of Development:** Phase 3 (Phase IIIb) **Medical Monitor:** Novartis Clinical Operations Lead

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the effectiveness of implementing a systematic LDL-C management pathway including treatment with inclisiran plus usual care, compared to usual care alone, on lowering LDL-C in patients who have experienced a recent acute coronary syndrome (ACS). **Secondary Objectives:** To assess the absolute change from baseline in LDL-C by visit, average percent and absolute changes in LDL-C, the proportion of patients reaching pre-specified LDL-C targets, modifications in other plasma lipids, adherence to background lipid-lowering therapies (LLT), and the overall safety and tolerability of inclisiran.

2.2 Background & Rationale Patients are at a high risk for recurrent cardiovascular (CV) events in the first year following an acute coronary syndrome (ACS). Low-density lipoprotein cholesterol (LDL-C) is a crucial modifiable risk factor. Despite the availability of statin therapies, many patients fail to achieve the guideline-recommended LDL-C target of < 70 mg/dL. To address this gap in post-ACS lipid management, the VICTORION-INCEPTION trial evaluates the early intensification of lipid-lowering therapy using inclisiran, a PCSK9-targeting small interfering RNA (siRNA), to provide rapid, strong, and sustained reductions in LDL-C.

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3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Usual Care) **Blinding:** Open-label **Randomization:** 1:1 (Parallel-group)

3.2 Planned Enrollment & Duration Targeted Enrollment: 400 participants (Approximately 384 participants planned for randomization) **Number of Sites:** Multicenter (US-based) **Estimated Study Start:** 24-Jun-2021 **Estimated Primary Completion:** 30-Aug-2024

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Recent Acute Coronary Syndrome (in-patient/out-patient) within 5 weeks of screening. 2. Serum LDL-C ≥ 70 mg/dL or non-HDL-C ≥ 100 mg/dL. 3. Fasting triglycerides < 4.52 mmol/L (< 400 mg/dL) at screening and calculated glomerular filtration rate > 20 mL/min by estimated glomerular filtration rate (eGFR). 4. Participants are required to be discharged on statin therapy, or have documented statin intolerance, as determined by the investigator, following hospitalization for an ACS. Statin intolerant participants are eligible if they had intolerable side effects on at least 2 different statins, including one at the lowest standard dose.

4.2 Exclusion Criteria 1. New York Heart Association (NYHA) class IIIb or IV heart failure, or last known left ventricular ejection fraction $< 25\%$. 2. Significant cardiac arrhythmia within 3 months prior to randomization that is not controlled by medication or via ablation at the time of screening. 3. Severe concomitant non-cardiovascular disease that carries the risk of reducing life expectancy to less than 2 years. 4. Treatment with other investigational products or devices within 30 days or five half-lives of the screening visit, whichever is longer. 5. Planned use of other investigational products or devices during the course of the study. 6. Treatment with monoclonal antibodies directed towards PCSK9 within 90 days of screening. 7. Recurrent ACS event within 2 weeks prior to randomization. 8. Coronary angiography and revascularization procedure (percutaneous coronary intervention (PCI) or coronary artery bypass grafting (CABG) surgery) performed within 2 weeks prior to the randomization visit or planned after randomization.

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Inclisiran sodium (Leqvio; ATC Code: C10AX16) 300 mg / 1.5 mL (equivalent to 284 mg of inclisiran). **Route of Administration:** Subcutaneous injection. **Duration of Treatment:** 1-year study duration (30-day screening period and a 330-day treatment period). **Product Reference:** EMA Product Information

5.2 Concomitant & Prohibited Therapy Permitted: Background standard of care (usual care) lipid-lowering therapies (LLT) including maximally tolerated statins, ezetimibe, or bempedoic acid, PCSK9-inhibiting monoclonal antibodies, and/or commercially available inclisiran at the discretion of the managing physician. **Prohibited:** Treatment with other investigational products/devices within 30 days or five half-lives of screening, planned use of investigational products during the study, or recent treatment with monoclonal antibodies directed towards PCSK9 (within 90 days).

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6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :--- | :--- | :--- | | **Screening** | Day -30 to 0 | Informed Consent, Medical History, Fasting Lipid Profile (LDL-C, Triglycerides) | | **Baseline** | Day 0 | Randomization (1:1), First Dose of Inclisiran (for intervention arm) | | **Interim** | Scheduled Visits | Follow-up LDL-C/Lipid profile assessments, Background LLT Adherence check, Safety Assessment | | **End of Study** | Day 330 | Final safety/ tolerability labs, Efficacy outcomes, Healthcare resource utilization evaluation |

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoints * **Percent Change From Baseline to Day 330 in LDL-C:** Percent change from baseline in Low-Density Lipoprotein Cholesterol (LDL-C) at Day 330. * **Achievement of LDL-C < 70 mg/dL at Day 330:** Percentage of participants achieving Low-Density Lipoprotein Cholesterol (LDL-C) < 70 mg/dL at Day 330. * **Measurement Method:** Central Laboratory Analysis of fasting lipid profiles.

7.2 Secondary & Exploratory Endpoints * Absolute Change From Baseline in LDL-C. * Average Percent Change From Baseline in LDL-C Levels. * Average Absolute Change From Baseline in LDL-C Levels. * Proportion of patients reaching pre-specified LDL-C targets and modifications in other plasma lipids. * Adherence to background LLT. * Safety and tolerability of inclisiran. * Exploratory: Patient-reported outcomes and healthcare resource utilization.

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via clinical visits (routine assessment of potential injection-site reactions and drug-related AEs) to assess safety and tolerability. **Serious**

Adverse Events (SAEs): Safety monitoring with standard expedited reporting timelines (typically 24 hours). **Data Monitoring Committee (DMC):** Internal/External safety overview adhering to standard Novartis operating protocols.

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9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy variables; Safety Set for tolerability and adverse event analyses. **Sample Size Justification:** Target enrollment of 400 participants (approximately 384 randomized) powered to detect a statistically significant distinction in LDL-C reduction between aggressive inclisiran pathway versus standard usual care. **Handling of Missing Data:** Standard methodologies (e.g., Mixed Model for Repeated Measures [MMRM] or Multiple Imputation) as dictated by the protocol's prespecified statistical model.

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Scheduled onsite and remote source data verification ensuring accurate lipid profile reporting and medication adherence. **Audit Trail:** Robust tracking of protocol deviations and standardized eCRF locking mechanisms.

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11. Study Identifiers & Governance

Primary ID: VICTORION-INCEPTION **Registry Number:** NCT04873934 **Sponsor/Lead Organization**: Novartis **Study Title:** VICTORION-INCEPTION Trial **Official Regulatory Title**: A Randomized, Controlled, Multicenter, Open-label Trial Comparing a Hospital Post-discharge Care Pathway Involving Aggressive LDL-C Management That Includes Inclisiran With Usual Care Versus Usual Care Alone in Patients With a Recent Acute Coronary Syndrome

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12. Clinical Framework (Cardiovascular Specific)

Primary Condition / Disease Area: Acute Coronary Syndrome (ACS) **Terminology Codes:** MeSH: D054058 | SNOMED CT: 394659003 | HPO: HP:0033678 **Diagnosis Threshold:** Serum LDL-C ≥ 70 mg/dL or non-HDL-C ≥ 100 mg/dL **Medical Methodology:** Systematic aggressive hospital post-discharge LDL-C management pathway **Optimization Target:** Achievement of LDL-C < 70 mg/dL

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13. Study Procedures & Methodology

Management Pathway: Aggressive post-discharge LDL-C reduction featuring inclisiran plus standard lipid-lowering care. **Education Protocol (HCP):** Implementation of systematic guidelines prioritizing rapid escalation of LLT post-ACS. **Education Protocol (Patient):** Counseling regarding the necessity of medication adherence and understanding CV risks. **Quality Audit Mechanism:** Healthcare resource utilization tracking and adherence monitoring of background therapies.

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14. Observation & Follow-Up Schedule

Total Duration: 360 days (30-day screening period and a 330-day treatment period) **Onsite Visit Cadence:** Guided by the trial protocol to mimic real-world clinical practice post-ACS discharge. **Remote Monitoring Frequency:** Routine adverse event collection periodically throughout the 1-year follow-up. **Checklist Review Interval:** Scheduled in sync with specific follow-up lipid analyses.

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15. Sign-off & Approval

Role	Name	Signature	Date
Principal Investigator	[Name]	____	[DD-MMM-YYYY]
Clinical Operations Lead	[Name]	___	[DD-MMM-YYYY]
Lead Statistician	[Name]	_____	[DD-MMM-YYYY]